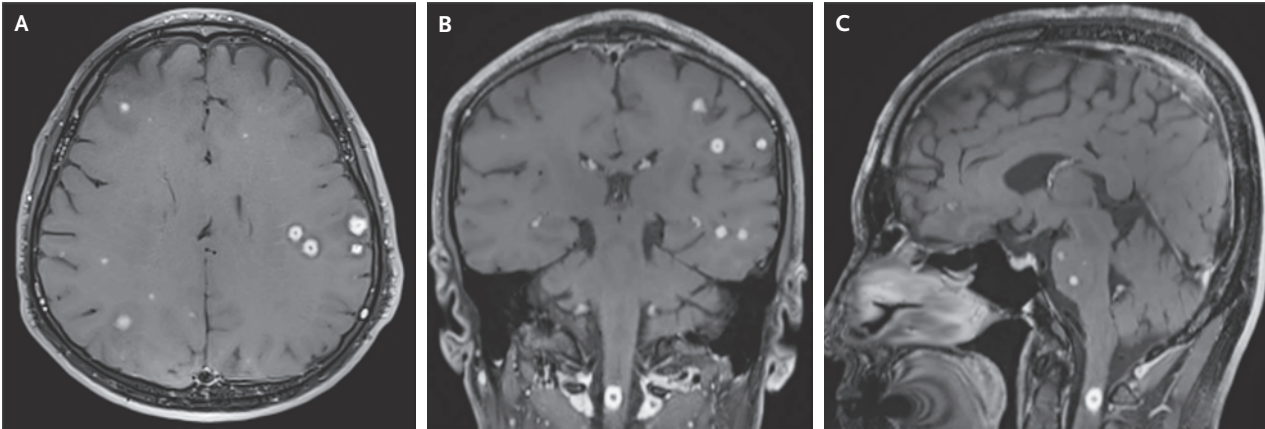


IMAGES IN CLINICAL MEDICINE

Central Nervous System Tuberculomas



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A 57-YEAR-OLD MAN WITH PREVIOUSLY TREATED PULMONARY TUBERCULOSIS presented to the emergency department with a 2-week history of neck pain, headache, and tingling in his right hand. Physical examination was notable for decreased grip strength in the right hand. Computed tomography of the chest revealed a miliary pattern of nodules in both lungs. Magnetic resonance imaging (MRI) of the head with gadolinium enhancement revealed numerous small, spherical, peripherally enhancing nodules in the cerebral hemispheres (Panels A and B), basal ganglia, cerebellum, and brain stem, as well as in the upper spinal cord with surrounding edema (Panel C). No leptomeningeal enhancement was noted. Results of cerebrospinal fluid analysis were normal, and cerebrospinal fluid cultures showed no growth. A sputum culture grew *Mycobacterium tuberculosis*. A diagnosis of central nervous system (CNS) tuberculomas in the context of miliary tuberculosis was made. Tuberculomas are granulomatous lesions that result from the hematogenous spread of tuberculosis. When many such lesions are present in the brain, a “starry sky” pattern may be seen on imaging. CNS tuberculomas may or may not (as in this case) be associated with tuberculous meningitis. Treatment with antituberculous agents and dexamethasone was initiated. Approximately 1 month later, the patient’s symptoms began to abate. At an 18-month follow-up visit, his symptoms had resolved completely, and repeat MRI of the head was normal.

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